

White Paper

The Path to CAR T-Cell Therapy

Uncovering barriers to patient access

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Table of contents

Introduction	1
Methodology	2
Sample	2
Overview of survey outputs	3
Barriers experienced at a referring center	4
Perceived patient ineligibility	4
Patient hesitancy	5
Coordination with ATCs	5
ATC uncertainties	5
Barriers experienced at ATCs	6
Poor patient health	6
Perceived CAR T-cell therapy risks	7
Perceived affordability challenges	7
Logistics	7
Priority solution areas	8
Referring oncologists	8
Treating centers and administrators	9
Patient advocacy groups	10
Pharmaceutical manufacturers	10
Conclusion	11
Appendix	12
References	14
About the authors	16

Introduction

CAR T-cell therapies generated intense anticipation ahead of their 2017 launch, offering a potentially transformative approach to combat cancers that experts had previously deemed incurable.¹ Since initial authorization, CAR T therapies have expanded rapidly across multiple hematologic malignancies, now with six FDA-approved products addressing a broad range of relapsed or refractory B-cell lymphomas, leukemias, and multiple myeloma (Appendix, Table 1).^{2,3,4} These CAR T therapies have demonstrated superior efficacy compared to standard-of-care treatments and typically acceptable risk-benefit profiles, prompting label updates that approved CAR T usage in earlier lines of therapy, and eliminated the FDA's Risk Evaluation and Mitigation Strategy (REMS) program.^{4,5}

In the United States, most patients begin their treatment journey at a practice likely located near their home. They are often managed by a general oncologist or hematologic oncologist at an institution without advanced CAR T-cell treatment capabilities. When these oncologists believe

their patient may benefit from CAR T therapy, they must refer the patient to an Activated / Authorized Treatment Center* (ATC). ATCs are equipped with the necessary infrastructure, have trained personnel, and have been onboarded by manufacturers to administer CAR T-cell therapy. At an ATC, a new healthcare provider (treater) is added to the patient's care team.

Despite the groundbreaking potential of CAR T-cell therapies in improving patient outcomes, access to these treatments remains limited. In 2024, IQVIA conducted research in a group of community oncologists and CAR T ATC treaters to identify the underlying barriers that have hindered patients' access to CAR T for the treatment of Large B-Cell Lymphoma (LBCL). This research uncovered a substantial disparity between the number of patients who may be eligible for the treatment and those who ultimately receive it. Addressing these obstacles is essential to bridge the divide between the transformative potential of CAR T-cell therapy and real-world impact on patients' lives.⁶



*An Activated / Authorized Treatment Center (ATC) is defined as a hospital that has the specialized infrastructure, training staffed, and regulatory certifications required to safely administer CAR T-cell therapy.

Methodology

To understand the obstacles throughout the treatment journey, IQVIA conducted a double-blind, LBCL-focused, and product-agnostic survey with 166 stakeholders across the provider landscape between April and May of 2024. This included referring oncologists, CAR T treaters, and CAR T support staff (e.g., office administrators, case managers, and nurse practitioners), who all play a critical role at various stages of the treatment journey.

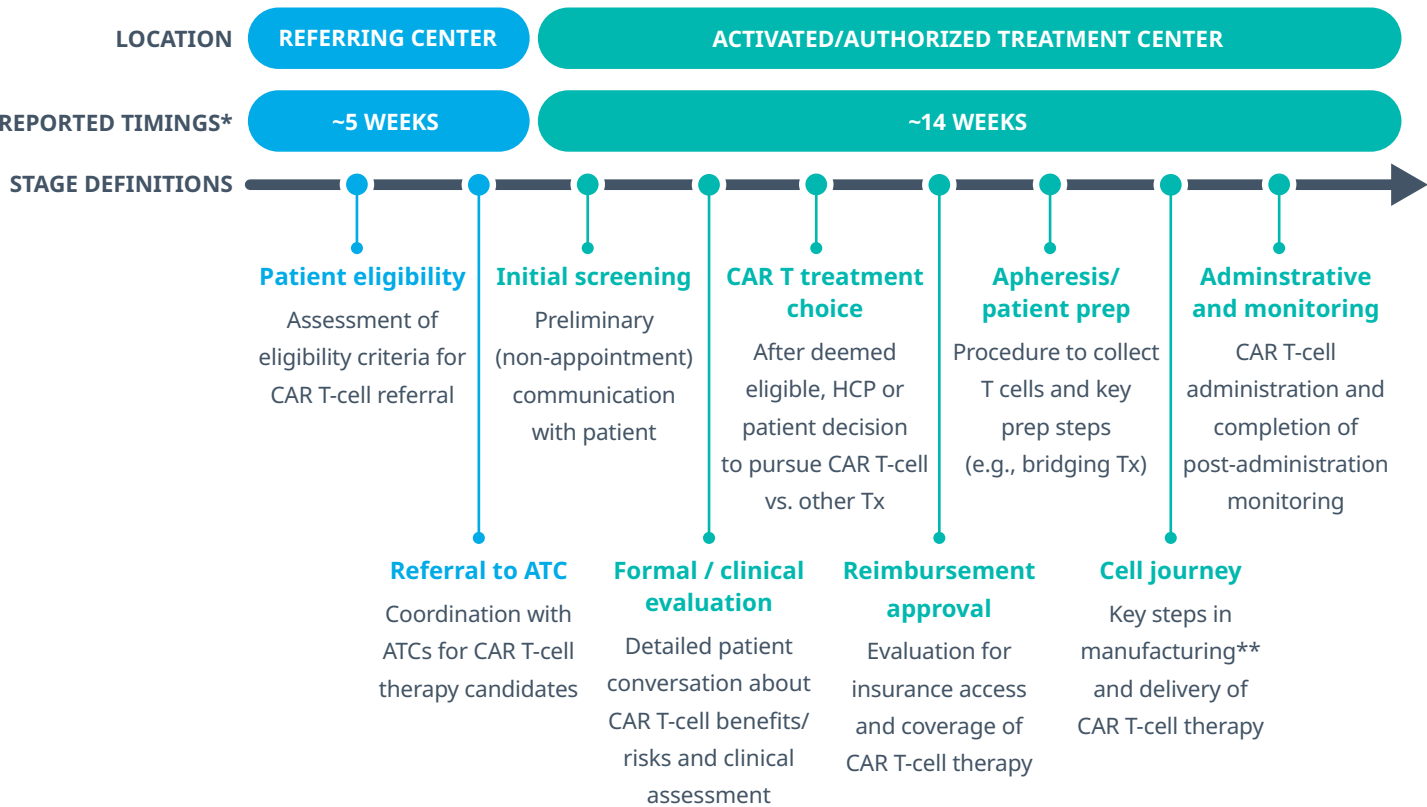
Sample

The referrers and CAR T treaters included in the sample were either hematologists or oncologists. Each stakeholder had between 3-30 years of experience and spent at least 50% of their time in direct patient care. IQVIA leveraged 2023 claims data to define high and low thresholds for referring oncologists and treaters in addition to triangulating referral and treatment volumes. Within the recruited sample,

respondents were classified as either high or low referrers based on the number of LBCL patients they had referred for CAR T-cell therapy in the three months prior to completing the survey. A high referrer is defined as an HCP who referred more than 10 patients to ATC in this period; a low referrer is an HCP who referred more than 1 and fewer than 10 patients to an ATC. IQVIA surveyed 22 high and 45 low referring oncologists.

CAR T treaters and support staff were defined as high if they had managed more than 20 LBCL patients with CAR T-cell therapy in the three months prior to participating in IQVIA's survey. A low treater was defined as someone who had managed at least one and fewer than 20 LBCL patients with CAR T-cell therapy in the past three months. The 2024 research sample included 34 high HCPs, 25 low HCPs, 16 high office administrators/nurse practitioners, and 24 low office administrators/nurse practitioners (Appendix, Table 2).

Figure 1: Key stages of the CAR T-cell therapy journey in LBCL, (n=166)



*Reported timings of each stage reflect insights from 2024 survey; **Manufacturing occurs offsite from the ATC

Overview of survey outputs

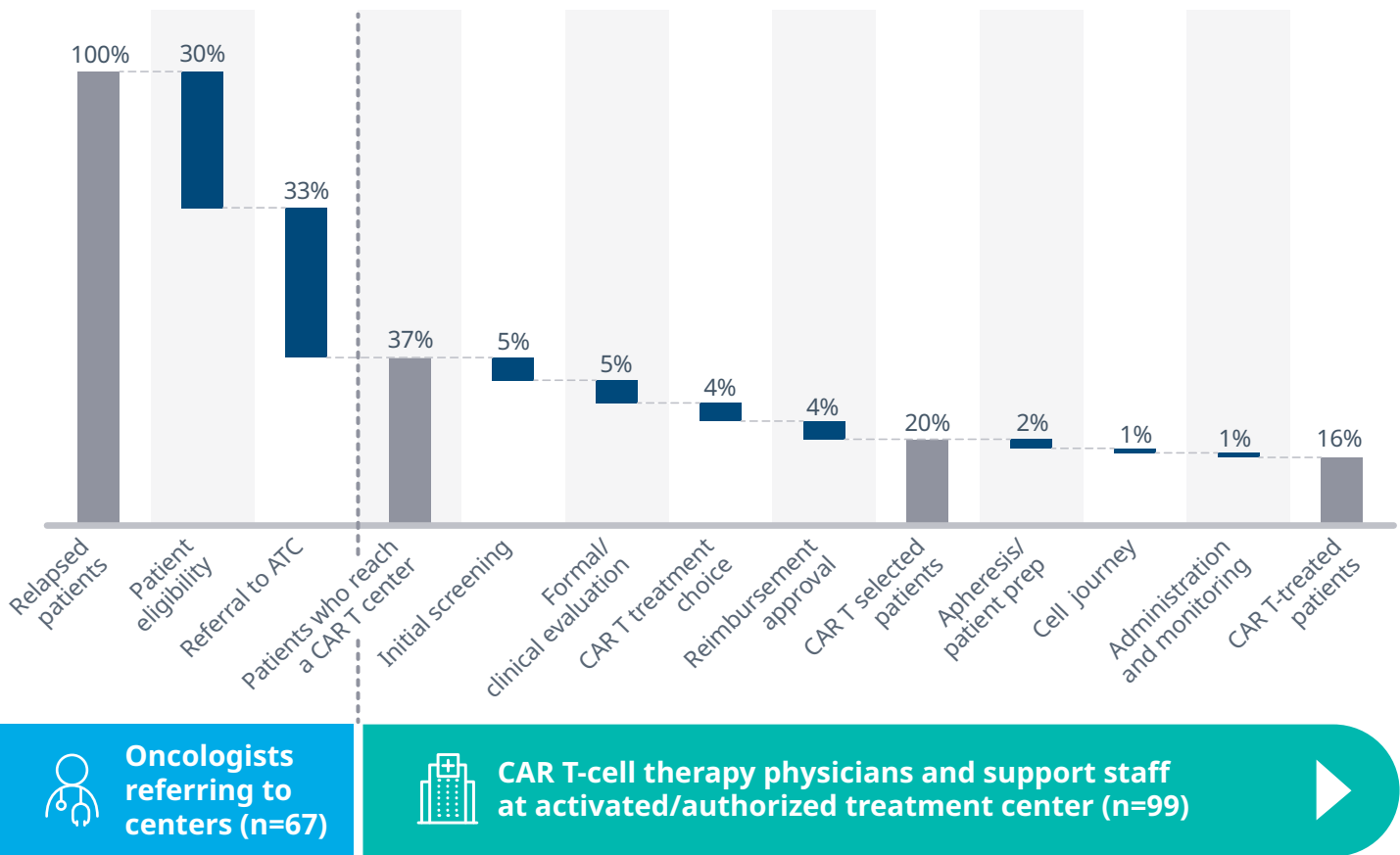
IQVIA's 2024 LBCL research identified and defined each stage of the CAR T therapy journey, including those that occur first in the community setting at a referring center and those that occur at an ATC. At a referring center, there are two sub-stages: Patient Eligibility and Referral to an ATC.

At an ATC, there are seven sub-stages, derived from previous IQVIA quantitative and qualitative research, beginning with the treating physician conducting an initial patient screening and continuing through the administration and monitoring stage (Figure 1).

The analysis also investigated where and why patients exit the treatment journey. As a result of the research findings, IQVIA developed the CAR T therapy waterfall (Figure 2), quantifying the barriers throughout the journey.

Surveyed stakeholders cited a range of barriers across the CAR-T treatment journey, including disease-related factors, patient-specific concerns, and logistical challenges that prevent eligible patients from accessing CAR T-cell therapy.⁶ Referring oncologists often failed to refer patients because of perceived ineligibility, patient hesitancy, or the complexity of coordinating with ATCs. Meanwhile, CAR T treaters reported that patients may drop off due to poor health status, perceived risks, perceived affordability concerns, or logistical issues.⁶ Upon analyzing the complete survey data, the findings revealed that almost two-thirds of eligible patients never progress through this initial stage. Moreover, the research indicated that overall, less than 2 out of 10 LBCL patients eligible for CAR T-cell therapy ultimately receive the treatment (Figure 2).

Figure 2: Patient attrition across the CAR T-cell therapy journey in LBCL: Waterfall analysis by stage



Barriers experienced at a referring center

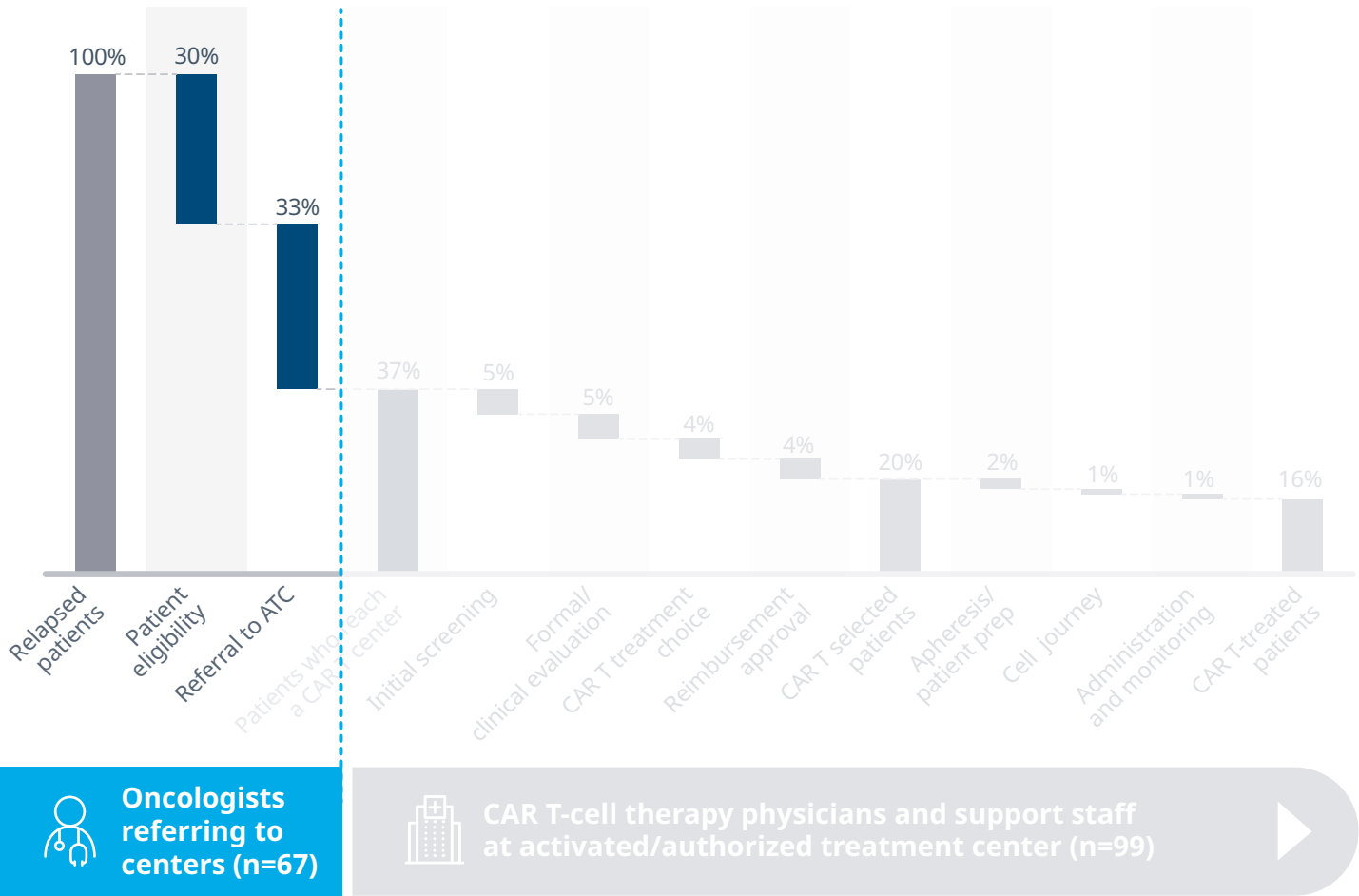
In the community setting, there exists an initial set of challenges that leads to almost two out of every three potentially CAR T eligible LBCL patients failing to reach an ATC (Figure 2a).⁶

Perceived patient ineligibility

Approximately 30% of the drop-off that occurs while in the referring setting can be attributed to perceived patient ineligibility. Referring oncologists perceived poor general health and poor Eastern Cooperative Oncology Group (ECOG) performance status (Appendix Table 3) as reasons why relapsed or refractory LBCL patients are ineligible for CAR T therapy. Interestingly,

there were clear differences in the criteria used by referrers for determining eligibility that was correlated to their amount of past experience referring for CAR T therapy. The surveyed physicians provided varied responses when evaluating age, comorbidity (number and type), and performance status thresholds for CAR T-cell therapy eligibility. For example, high referring oncologists report referring older LBCL patients (ages 80+) for treatment. Oncologists who have less comfort with CAR T-cell therapy, however, tend to apply lower age cutoffs and are more likely to refer only younger (<70 years of age).⁶ It is also important to note that FDA labels do not specify an upper age limit. Finally, there is some real-world data that have indicated that older and frailer patients treated with CAR T have similar outcomes to the overall population.^{7,8,9}

Figure 2a: Patient attrition across the CAR T-cell therapy journey LBCL: referral focus





While researchers developed the ECOG performance status scale to standardize criteria for measuring how cancer impacts a patient's daily living abilities, surveyed physicians do not apply limits consistently when using it to determine patient eligibility for CAR T therapy.¹⁰ IQVIA's 2024 LBCL research found that 46% of surveyed referring oncologists used ECOG ≥ 2 as a cutoff threshold for CAR T referral eligibility, while 38% used ECOG ≥ 3 . While a higher ECOG score signals reduced physical fitness, the difference in utilization indicates that physicians use ECOG subjectively.⁶

Patient hesitancy

Among the 33% of patients that drop-off from the CAR T therapy journey during the Referral to an ATC stage (Figure 2a), 18% of those departures include eligible patients that ultimately decline the treatment, not because of clinical concerns, but because they are uncertain of what the journey requires or believe it to be too burdensome. The CAR T-cell therapy process typically occurs over several weeks and requires multiple visits to an ATC, leading some patients to determine the time, travel, and costs associated with treatment too great a burden to undertake. The referring oncologists surveyed confirmed this perception among patients; their responses show that approximately 35% of patients underestimate the therapeutic benefits of CAR T-cell therapy despite the fact that recent trials show significant complete response rates.

Coordination with ATCs

Referring oncologists also reported that approximately 15% of eligible LBCL patients do not proceed with CAR T therapy due to challenges coordinating with ATCs.⁶ In many cases, referrers directly cited that drop-off occurs because of lack of responsiveness from ATCs as the barrier to continuing with the referral.

ATC uncertainties

The uncertainties perceived by the referring oncologists surrounding the logistics and timing of CAR T therapy administration at ATCs, particularly regarding scheduling and CAR T manufacturing time, are undermining patient access to this potentially life-saving treatment. The data collected from surveyed referring oncologists, for example, showed a perception of limited scheduling times for CAR T therapy at ATCs, and that these slots are preferentially assigned to those already affiliated with the center; ~5% of patients drop-off during this stage because of this misconception.⁶ Industry analysts clarify this perception and suggest that it likely stems from the manufacturing issues that accompanied CAR T-cell therapy when these products were first launched in 2017. Researchers at Cowen, a financial services firm, have noted in their publications that physicians and patients still view manufacturing time as a limiting factor despite the improved process efficiencies in recent years.⁴ While the introduction of additional manufacturing sites has reduced production time and increased slot availability, the 2024 LBCL research confirms that some referring oncologists still associate treatment postponements with CAR T-cell therapy.⁶

Barriers experienced at ATCs

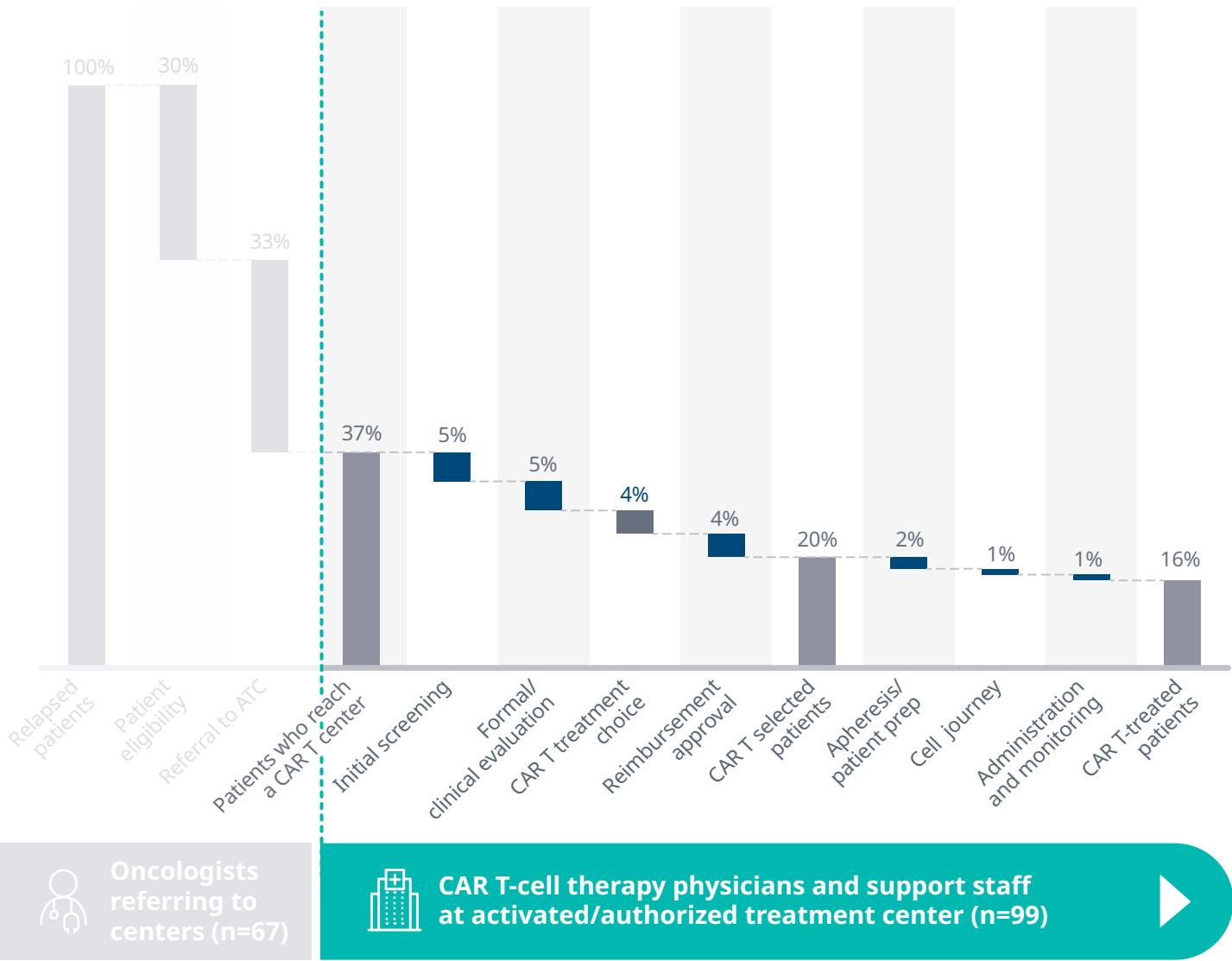
Barriers exist for the LBCL patients who are successfully referred and reach an ATC, with approximately half of these patients exiting the treatment journey before they progress to the apheresis/patient prep stage (Figure 2b).⁶

Poor patient health

Nearly one in four patients who reach a CAR T center fall out of the process after the Initial Screening or Formal / Clinical Evaluation stages as the treating physician at the ATC deems them ineligible. While the precise reason is difficult to determine, IQVIA's 2024 LBCL research

suggested potential causes. One explanation is that the referring oncologist provides an open referral, but the treating physician at the ATC determines that CAR T therapy is not the optimal form of treatment. It is also possible that patients who were in good health at the time of their referral may have deteriorated by the time they reach an ATC and are no longer eligible.⁶ While patients are ideally able to transition efficiently through the CAR T treatment journey, real-world data illustrate that variation in administration processes and capacity among ATCs can extend the waiting period.¹² If a patient's health diminishes throughout this time, they may no longer be able to tolerate CAR T-cell therapy and drop out of the process.

Figure 2b: Patient attrition during the CAR T-cell therapy journey in LBCL: ATC focus



Perceived CAR T-cell therapy risks

Another contributing factor to attrition at an ATC is preference for alternative treatment options, largely due to patient perception of the overall CAR T-cell therapy risk-benefit profile. CAR T-cell therapy can cause such side-effects such as Cytokine Release Syndrome (CRS), Immune Effector Cell-Associated Neurotoxicity Syndrome (ICANS), and infections.¹³ The data collected from surveyed CAR T treaters showed that ~10% of LBCL patients opt for other treatment options at this stage due to concern about these side-effects. Additionally, ~50% of patients who reach this stage may perceive alternative treatments, as effective and less risky compared to CAR T-cell therapy.⁶ In the time since CAR T therapy entered the market, clinical trials and real-world evidence have demonstrated that the potential risks of CAR T-cell therapy can be managed effectively. As data confirm that most patients recover well from CAR T-cell therapy side effects and that the risk of developing CRS / ICANS is mostly limited to the first two weeks post infusion, the FDA removed the REMS safety program and reduced the monitoring requirements for CAR T-cell therapy from one month to two weeks as of June 2025.^{5,14} This shift in guidance will likely increase access to CAR T-cell therapy and have a positive effect on the patient experience following treatment administration, as it allows patients to significantly decrease the time they are required to stay close to an ATC.¹⁴

Perceived affordability challenges

Patients' and treaters' apprehension surrounding perceived treatment affordability may also derail the CAR T therapy journey. Surveyed respondents reported that ~43% of patients have concerns about the potential high out-of-pocket costs and gaining insurance approval. CAR T treaters reported that ~11% of LBCL patients who reach an ATC leave the CAR T process before undergoing apheresis due to perceived unaffordable costs or delays in insurance approval. The research also showed differences in the extent and type of affordability challenges reported between large and small centers.

Larger centers reported fewer challenges with affordability, as they are typically more familiar with the insurance processes and financial assistance programs available for CAR T-cell therapy and, therefore, are better equipped to transition patients through this stage efficiently.⁶ Ideally, financial apprehension should not inhibit patient access to efficacious treatments like CAR T-cell therapy.

Logistics

Finally, even for patients who are eligible and motivated to receive CAR T-cell therapy, the logistical requirements of administration can become insurmountable barriers. According to the IQVIA research, ~5% of patients do not move forward in the CAR T-cell therapy journey for this reason. One cause for the logistical barriers is that treaters typically require patients to stay a close distance from their ATC post-infusion to enable monitoring and follow-up visits, which may result in significant time and costs. Additionally, treaters encourage patients to have a dedicated caregiver present for the duration of safety and monitoring after infusion, which can impose further financial and scheduling strain.^{5,15} The fact that the FDA eliminated the CAR T-cell REMS program in June 2025 and reduced monitoring requirements for patients who have received treatment may alleviate some of these concerns.⁵

Collected data from surveyed physicians confirmed that patients also may be concerned about potential delays in receiving their treatment because of the extended administrative processes required before infusion. IQVIA's research found that the average time from initial stage of the CAR T treatment journey to the infusion process occurs over ~14 weeks (Figure 1).^{6,12} Administrative hurdles, like delays in scheduling appointments, inefficiencies in processing required documentation, and impediments in authorization delays are the main reasons for these extended timelines. This waiting period can cause patients who were initially eligible to deteriorate in health and become ineligible for CAR T-cell treatment.^{6,16}

Priority solution areas

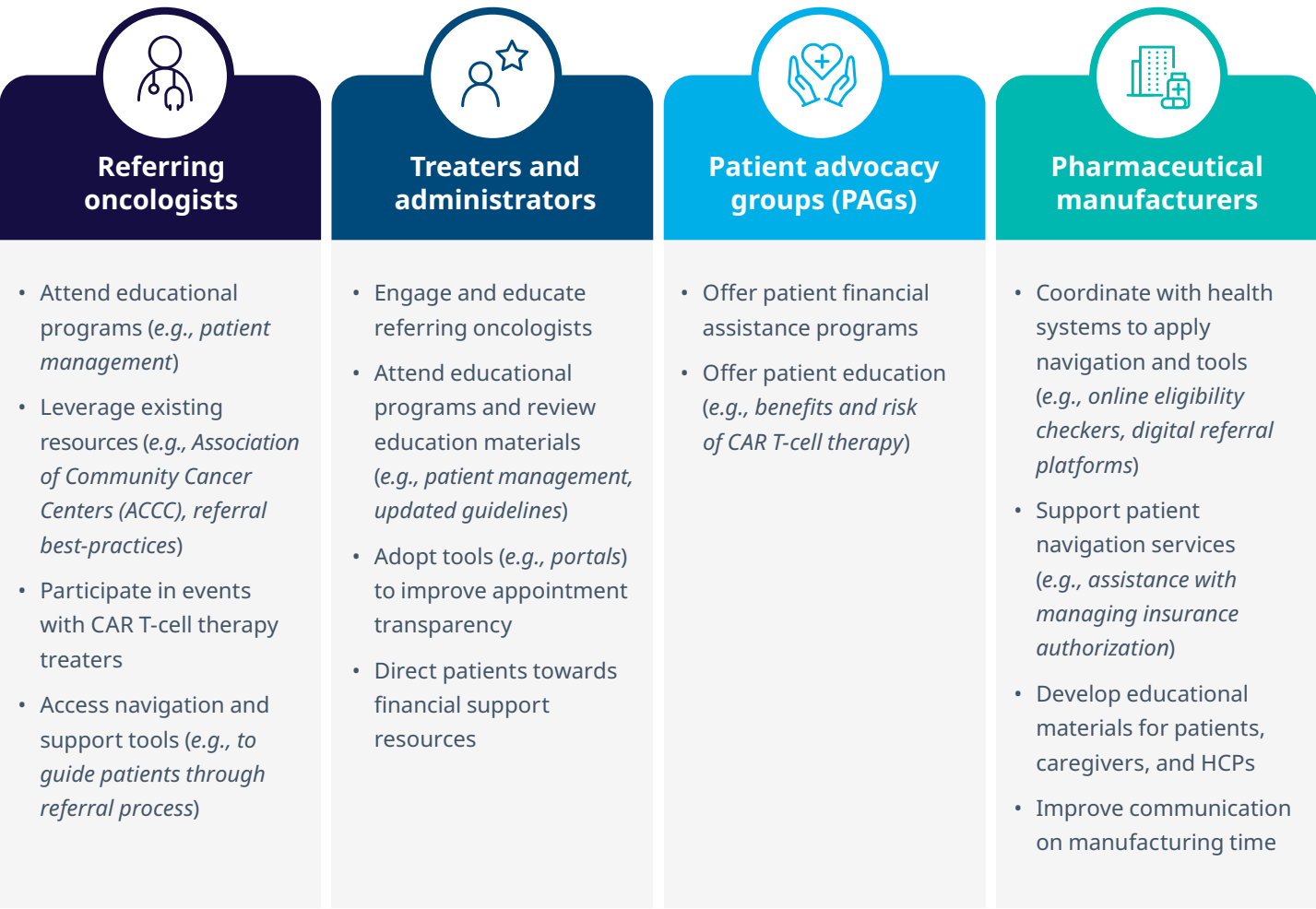
As part of IQVIA's 2024 LBCL research, referring oncologists and treaters evaluated potential solutions to address existing barriers to CAR T-cell therapy. Survey participants assessed a range of interventions that could be implemented across the treatment journey and rated each one on a scale of one to seven, where one is no impact in addressing CAR T drop-off and seven is high impact in addressing CAR T drop-off. While patient advocacy groups (PAGs), patients, and manufacturers were not included in the sample, referring oncologists and treaters offered insights into concepts that these groups can adopt. Respondents recognized that improving access to treatment will require coordinated efforts across all key stakeholders involved in the CAR T process. Solutions can

be grouped by priority stakeholders (referring oncologists, treaters, PAGs, and pharmaceutical manufacturers) with such key themes as enhancing healthcare education, expanding financial support programs, and streamlining care coordination — across all groups (Figure 3).⁶ These strategies aim to reduce uncertainty around CAR T therapy, empower stakeholders to better support patients, and promote equitable access to care.

Referring oncologists

As the referring stage of the process comprises the largest points of drop-off to CAR T-cell therapy, referring oncologists are critical to help drive change. The surveyed physicians rated increased education as well as alignment on patient eligibility and referral best practices as a particularly impactful way to reduce barriers, with a score

Figure 3: Potential stakeholder solution areas for improving access to CAR T-cell therapy



of 5.0 out of 7.⁶ Their rationale stems from the fact that referring oncologists are the stakeholders who will first introduce the prospect of CAR T therapy to patients. Moreover, referring oncologists will continue to care for patients after CAR T therapy has been administered, making it critical for them to be well educated about the treatment. To access educational materials, referring oncologists can participate in medical association and pharmaceutical company programs as well as leverage such existing resources as patient/caregiver education and coordination tools.^{17,18} Respondents also saw value in strengthening the peer-to-peer relationships between referring oncologists and treaters, rating this concept with a score of 4.8.⁶ Attending educational events with CAR T treaters and ATCs can further support relationship building and awareness of support tools (e.g., referral navigators).¹⁹ Enhancing referring oncologists' engagement with educational programs, optimizing their use of available resources, and fostering participation in collaborative events with treaters can strengthen the referral process and reduce the number of eligible patients overlooked for CAR T therapy.

Treaters and administrators

Treaters are also uniquely positioned to bridge the gap between innovation and access. The survey data show that they place high value on targeted solutions that foster stronger collaboration with referring oncologists, streamline referral logistics, and guide patients through financial and clinical complexities, with high ratings from respondents (5+ out of 7). Treaters recognize that they can play a pivotal role in educating their referring oncologist colleagues to ensure alignment on patient eligibility for the CAR T-cell therapy process, thereby facilitating broader access to this innovative treatment. Their survey responses show, with a rating of 5.0, that actively encouraging referring oncologists to participate in educational programs related to CAR T-cell therapy and to take responsibility for fostering collaboration through seminars, meetings, and shared

learning opportunities would also be impactful. Treaters illustrated the value they placed on remaining current with treatment guidelines and clinical data by assigning a similar average rating of 5.0 to reduce unnecessary drop-off from the CAR T-cell process.⁶

To improve transparency around appointment availability at ATCs, survey respondents acknowledge the value of treaters and their hospital system administration implementing standardized referral frameworks, such as the ASTCT/ACCC/AACI's RECUR* to promote early patient identification and streamlined referral workflows; respondents rated this solution with a 5.1 on average.^{18,20} Treatment centers can also leverage internal electronic systems that provide referring oncologists with access to appointment calendars or scheduling portals to improve direct communication channels on appointment availability.⁶

Survey respondents identified increasing patient and caregiver awareness of available financial support programs as among the most impactful of all proposed solutions. Treaters appreciate the critical role that they can play to help CAR T-cell therapy eligible patients navigate any financial concerns. Physicians report that many patients lack awareness of available support services and of financial coordinators who can assist with insurance verification, cost estimation, and identifying coverage gaps. Treaters can provide patients with materials about grants and nonprofit programs, such as the *Leukemia & Lymphoma Society's Susan Lang Pre-CAR T-cell Therapy Travel Assistance Program*, which provide patients with travel and lodging during the treatment evaluation phase.²¹ Similarly, they can direct patients to support programs provided by pharmaceutical manufacturers that offer patient navigation, financial assistance, and logistical support.⁶ By fostering stronger collaboration with referring oncologists, streamlining referral logistics, and guiding patients through financial and clinical complexities, treaters can help ensure CAR T-cell therapy reaches a broader patient population.

*RECUR is the abbreviation for Relapsed and refractory large B cell lymphoma, Every age and comorbidity, Caregiver support, Urgency to recommend consult, and Receive patients who return post CAR T-cell therapy

Patient advocacy groups

To further improve the CAR T-cell therapy experience and broaden its availability for patients, the surveyed physicians assessed the impact of solutions targeted for PAGs. PAGs can expand their role in two key areas: financial assistance and patient education. First, PAGs can help to create more robust and scalable financial assistance programs that go beyond one-time grants, such as creating pooled funds, partnering with philanthropic organizations, or advocating for dedicated public subsidies to reduce the financial burden on patients. PAGs can also direct patients towards existing organizations that offer financial support, including the Leukemia & Lymphoma Society (now Blood Cancer United), Patient Access Network, and HealthWell Foundation. These groups have multiple assistance programs that can help patients with affordability challenges ranging from coverage gaps to out-of-pocket costs and inpatient stays.^{22,23,24} Second, PAGs can invest in comprehensive, multilingual education campaigns that proactively inform patients and caregivers about the full spectrum of assistance options available, including how to navigate complex insurance systems and appeal denials.⁶ By focusing on expanding financial assistance and patient education, PAGs can help address major barriers to CAR T-cell therapy and improve treatment accessibility.

Pharmaceutical manufacturers

The 2024 LBCL research shows that physicians appreciate the unique opportunity manufacturers have to directly address the perceived operational barriers that impact CAR T-cell therapy accessibility. Offering solutions that streamline the referral process (score of 5.0), enhance patient and provider education (score of 5.0), and improve transparency about manufacturing time (score of 5.3) were perceived as valuable by the respondents. To help manage complex steps like insurance authorization and scheduling, the surveyed physicians suggested that manufacturers could provide logistical assistance and patient navigation services.^{6,25} Key stakeholders would appreciate if manufacturers could also improve provider and patient understanding of CAR T-cell therapy by offering educational resources and decision aids about the treatment process. These resources include guidelines on eligibility criteria, summaries of the benefits and risks of CAR T-cell therapy, recommendations for patient management following CAR T-cell infusion, and an outline of the CAR T-cell journey to set HCP, patient, and caregiver expectations for each step.⁶ Finally, by offering treatment tracking tools, manufacturers can provide HCPs and patients visibility into real-time updates on CAR T-cell delivery timelines, identifying centers



with capacity, and reducing time between referral and treatment. Clearer communication and better education were confirmed as high impact solutions that can dispel perceptions of persistent delays that have accompanied CAR T-cell production. Ultimately, through these efforts, manufacturers can play a pivotal role in expanding access to CAR T therapy by ensuring that all eligible patients, regardless of whether they initiate their treatment, receive timely and potentially life-saving treatment.

Conclusion

The results of the IQVIA commissioned research on the challenges to uptake of CAR T-cell therapies are striking. Two out of three eligible patients (relapsed or refractory LBCL) are never referred to a treatment center and less than two out of 10 patients ultimately receive CAR T-cell therapy treatment.⁶ These findings highlight the significant gulf between the clinical potential of CAR T-cell therapy and the real world.

Improving coordination among stakeholders across the healthcare ecosystem is an acknowledged high-impact solution to overcoming barriers throughout the CAR T therapy process. Ongoing education that improves referrer familiarity with CAR T-cell therapy eligibility criteria, encourages referral intention, and teaches post-treatment care are key initial steps to improving access. Enhanced collaboration between referrers and treaters will also positively impact the referring process by building trust and streamlining communication. And accessibility to up-to-date information on CAR T-cell therapy guidelines and long-term clinical outcomes will help both treaters and referrers improve alignment on patient suitability.

Additionally, treaters should encourage their ATCs to implement standardized referral frameworks that improve appointment availability visibility and create new roles dedicated to managing referral logistics. Collected responses from the 2024 research emphasized the

importance of treaters clearly communicating financial support options to ensure that perception of unaffordability does not cause patients to drop off. PAGs can expand their impact by strengthening financial assistance programs and launching multilingual education campaigns.

Enhancing education for patients while they are at a referring center and an ATC will play a critical role in bridging the gap between eligibility and access to CAR T-cell therapy. By informing patients with clear, accessible information about treatment options, referral pathways, and potential but manageable side effects, healthcare providers can reduce delays, improve decision-making, and foster greater engagement throughout the care journey.

Lastly, CAR T-cell manufacturers can also help streamline the referral and treatment process by offering digital referral tools, eligibility checkers, and patient navigation services to physicians and patients to simplify access. Tracking platforms and transparent communication can help physicians and patients better understand manufacturing timelines and avoid perceived delays.

By investing in education, communication, financial resources, and logistical support, stakeholders can collectively reduce CAR T-cell therapy drop-off, improve the patient experience, and expand access to life-saving CAR T-cell therapies.

Appendix

Table 1: CAR T-cell therapy approved treatments³

PRODUCT NAME	MOLECULE NAME	MANUFACTURER	APPROVED INDICATIONS	APPROVAL YEAR	LINE OF THERAPY (LATEST APPROVAL)
Yescarta	Axicabtagene ciloleucel	Kite Pharma (Gilead Sciences) [†]	Large B-cell Lymphoma	October 2017	2L
			Follicular Lymphoma	March 2021	3L+
Kymriah	Tisagenlecleucel	Novartis	B-cell Precursor Acute Lymphoblastic Leukemia	August 2017	2L+
			Large B-cell Lymphoma	May 2018	3L+
			Follicular Lymphoma	May 2022	3L+
Tecartus	Brexucabtagene autoleucel	Kite Pharma (Gilead Sciences) [†]	Mantle cell Lymphoma	July 2020	R/R*
			B-cell Precursor Acute Lymphoblastic Leukemia	October 2021	R/R*
Breyanzi	Lisocabtagene maraleucel	Bristol Myers Squibb	Large B-cell Lymphoma	February 2021	2L+
			Chronic/ small lymphocytic leukemia	March 2024	3L+
			Follicular Lymphoma	May 2024	3L+
			Mantle cell Lymphoma	May 2024	3L+
Abecma	Idecabtagene vicleucel	Bristol Myers Squibb and 2seventy bio	Multiple Myeloma	March 2021	3L+
Carvykti	Ciltacabtagene autoleucel	Legend Biotech and Janssen Biotech	Multiple Myeloma	February 2022	2L+

*R/R = Relapse Refractory. [†]Kite was acquired by Gilead in 2017 (Gilead News)

Appendix (cont'd)

Table 2: IQVIA 2024 research definitions and sample overview⁶

	DEFINITION	SAMPLE OVERVIEW
High referrer	Referred >10 patients to an ATC for CAR T-cell therapy in the past 3 months	N = 22
Low referrer	Referred 1-10 patients to an ATC for CAR T-cell therapy in the past 3 months	N = 45
High volume treaters	Evaluated >20 patients for CAR T-cell therapy in the past 3 months	N = 50
Medium/Low volume treaters	Evaluated 1-20 patients for CAR T-cell therapy in the past 3 months	N = 49
Academic setting	• An academic center or teaching hospital • Group practice owned by an academic hospital	Referring oncologists: N = 34 • High: 13 • Low: 19 Treators: N = 55 • High: 22 • Low: 33
Community setting	• A non-teaching hospital or community hospital • A group practice owned by a community hospital • Private practice not owned by a hospital	Referring oncologists: N = 33 • High: 9 • Low: 26 Treators: N = 44 • High: 28 • Low: 16
Primary medical specialty	Specialty indicated by respondent as primary focus area	Referring oncologists: • Hematologists: N = 43 ◦ High: 17 ◦ Low: 26 • Medical oncologists: N = 24 ◦ High: 5 ◦ Low: 19

Table 3: Eastern Cooperative Oncology Group (ECOG)* performance status definition and scale¹⁰

OG SCALE	DESCRIPTION
0	Fully active, able to carry on all pre-disease performance without restriction
1	Restricted in physically strenuous activity but ambulatory and able to carry out work of a light or sedentary nature
2	Ambulatory and capable of self-care but unable to work activities; up and about more than 50% of waking hours
3	Capable of only limited self-care; confined to a bed or chair more than 50% of waking hours
4	Completely disabled; cannot carry on any self-care; totally confined to a bed or chair
5	Deceased

*ECOG Status Definition: A standardized scale used in oncology to assess a cancer patient's level of functioning and ability to carry out daily activities. It is commonly used to guide treatment decisions and eligibility for clinical trials.

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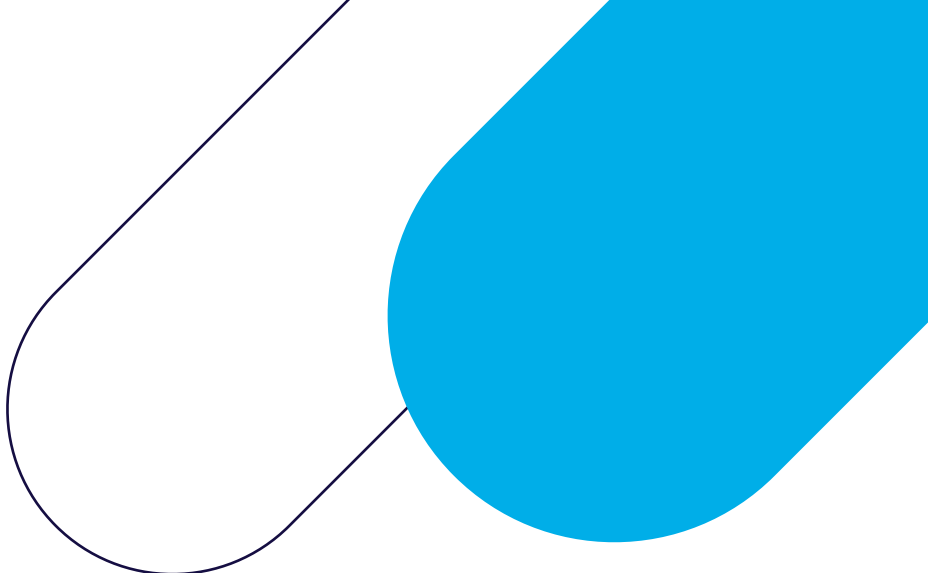
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